



RadOn Cancer Centre

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Laboratory Report

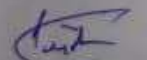
Name : Mr. KAMALARKAR G HEGDE
Age/Gender : 83 Y/M
Sample ID : BL020782
Admitting Doctor : Dr. MEHAR KUMAR
Rate Plan : GENERAL

MR No : RCC006934
Visit ID : OP25006193
Sample Date : 14-08-2025 12:34
Report Date : 14-08-2025 12:34

HAEMATOLOGY

Test Description	Methodology	Result	Units	Ref Range
Sample Type: BLOOD				
CBC WITH PLATELET COUNT				
HEMOGLOBIN		14.9	g/dL	13-16
RBC		4.82	millions/cmm	4.5-5.5
TOTAL WBC COUNT		7,190	cells/cumm	4000-11000
POLYMORPHS		91 *	%	50-70
LYMPHOCYTES		04 *	%	20-40
EOSINOPHILS		02	%	1-4
MONOCYTES		03	%	2-10
BASOPHILS		00	$\times 10^4/L$	0-1
PLATELET COUNT		1.85	Lakhs/Cumm	1.5-4.5
HCT/PCV RED CELL INDICES		42.9	%	35-55
M.C.V		88.9	fl	75-100
M.C.H		30.9	pg	25-35
M.C.H.C		34.8	%	31-37

End of the report


Lab Technician

INAYAT

Pathology
DR. JATESH

A unit of Radon Oncology Pvt. Ltd.

#85, Javali Garden, M.T. Sagar, Off Gokul Road, Hubballi- 580030 Tel: 0836-2335925/2239600
CIN : U85100KA2012PTC065541

Verified By:



RadOn Cancer Centre

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Laboratory Report

Name : Mr. KAMALARKAR G HEGDE
Age/Gender : 83 Y/M
Sample ID : BL020639
Admitting Doctor : Dr. MEHAR KUMAR
Rate Plan : GENERAL

MR No : RCC006934
Visit ID : OP25005863
Sample Date : 30-07-2025 13:19
Report Date : 30-07-2025 13:20

HAEMATOLOGY

<u>Test Description</u>	<u>Methodology</u>	<u>Result</u>	<u>Units</u>	<u>Ref Range</u>
Sample Type: BLOOD				
CBC WITH PLATELET COUNT				
HEMOGLOBIN		14.6	g/dL	13-16
RBC		4.77	millions/cmm	4.5-5.5
TOTAL WBC COUNT		6,070	cells/cumm	4000-11000
POLYMORPHS		60	%	50-70
LYMPHOCYTES		20	%	20-40
EOSINOPHILS		13 *	%	1-4
MONOCYTES		07	%	2-10
BASOPHILS		00	$\times 10^9/L$	0-1
PLATELET COUNT		2.88	Lakhs/Cumm	1.5-4.5
HCT/PCV RED CELL INDICES		41.9	%	35-55
M.C.V		87.9	fL	75-100
M.C.H		30.6	pg	25-35
M.C.H.C		34.8	%	31-37

End of the report

Lab Technician

INAYAT

P.
DR. JAT

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CIN : U85100KA2012PTC065541



HUBLI SCAN CENTRE

6-13, LGF, Eureka Junction, Travellers Bungalow Road, HUBLI - 580 029.

☎: 0836-2257828
2257838
2257848
2228278

Patient Name:	KAMALAKAR HEGDE 83y/M	Patient ID:	118689
Age:	83 Years	Sex:	M
Study Date:	28-Jul-2025	Referred By:	Dr. VISHWAS D. PAI
			M.S., M.Ch., DNB(Onco)

-2-

Emphysematous changes in both lungs noted.
Fibrotic strands and plate atelectasis noted in bilateral upper, lower, right middle and lingular lobe.
Dependant densities are noted in bilateral lower lobes.
A small calcification is noted in right upper lobe of lung.
Rest of bilateral lung parenchyma is clear. No evidence of infiltrates / nodule / mass.
Tracheo-bronchial tree is normal.
Pleura and fissures are normal.
No evidence of pleural effusion / thickening.
Atherosclerotic changes noted in the arch of aorta.
No significant mediastinal / hilar lymphadenopathy.
Chest wall structures are normal.
Covered portions of liver and spleen show no focal lesion.
Adrenals are normal in size.
Tiny ill-defined non-specific lucencies are noted in cervical and dorsal vertebral bodies and L1 vertebra.

Impression.

A large ill-defined enhancing ulceroproliferative mass lesion in right side of base of tongue. Lesion is extending into right vallecula and right tonsillar fossa. Lesion is involving left side of base of tongue, median and right glossoepiglottic folds, right side of epiglottis, pre-epiglottic space, right aryepiglottic fold and right lateral pharyngeal wall. Lesion is abutting right cornu of hyoid bone. No bony erosion seen. Focal extension into left aryepiglottic fold also noted. Narrowing of airway is noted. No extension into pre-vertebral space noted.

Multiple enlarged metastatic lymph nodes in right level II, III and V B. Largest lymph node measuring 4.5 x 3.3 cm is noted in right level II and parapharyngeal space. It is encasing right internal carotid artery and external carotid artery and its branches causing luminal narrowing. Severe compression of right internal jugular vein noted at the level of right level II and III lymph nodes.

Small lymph nodes in left level II. They are most likely reactive in nature.

No lung or liver metastasis seen.

Eccentric soft plaque in right carotid bulb and at the origin of right internal carotid artery causing 70% stenosis.

Emphysematous changes in both lungs.

Handrakant S. Katwe
M.D., D.M.R.D.
Consultant Radiologist

Dr. Rajesh S. Revankar
D.M.R.E., D.N.B.
Consultant Radiologist

Dr. Mohan S. Bhavikatti
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Dr. Rajeev B. Dibbad
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Consultant Radiologist

Dr. Raghavendra Bhat
D.M.R.D., M.D.(Med)
Consultant Radiologist

27 3700



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Laboratory Report

Name : Mr. KAMALARKAR G HEGDE
Age/Gender : 83 Y/M
Sample ID : SE019614
Admitting Doctor : Dr. MEHAR KUMAR
Rate Plan : GENERAL

MR No : RCC006934
Visit ID : OP25005863
Sample Date : 30-07-2025 13:19
Report Date : 30-07-2025 13:20

BIOCHEMISTRY

Test Description	Methodology	Result	Units	Ref Range
Sample Type: SERUM				
LFT (LIVER FUNCTION TEST)				
BILIRUBIN TOTAL		0.7	mg/dL	0.2-1.5
BILIRUBIN DIRECT		0.3	mg/dL	0.0-0.3
BILIRUBIN INDIRECT		0.4	mg/dL	0-1.1
AST/SGOT		14	U/L	5-30
ALT/SGPT		13	U/L	5-35
ALKALINE PHOSPHATASE		81	U/L	45-116
TOTAL PROTEINS		6.6	g/dL	6.02-8.0
ALBUMIN		3.5 *	g/dL	3.7-5.6
GLOBULINS		3.1 *	g/dL	2.5-3.0

End of the report

Lab Technician

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Laboratory Report

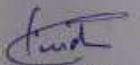
Name : Mr. KAMALARKAR G HEGDE
Age/Gender : 83 Y/M
Sample ID : SE019770, SE019770
Admitting Doctor : Dr. MEHAR KUMAR
Rate Plan : GENERAL

MR No : RCC006934
Visit ID : OP25006193
Sample Date : 14-08-2025 12:34
Report Date : 14-08-2025 12:35

BIOCHEMISTRY

Test Description	Methodology	Result	Units	Ref Range
Sample Type: SERUM				
SERUM CREATININE		1.2	mg/dl	0.8-1.4
Sample Type: SERUM				
LFT (LIVER FUNCTION TEST)				
BILIRUBIN TOTAL		0.7	mg/dL	0.2-1.5
BILIRUBIN DIRECT		0.4 *	mg/dL	0.0-0.3
BILIRUBIN INDIRECT		0.3	mg/dL	0-1.1
AST/SGOT		13	U/L	5-30
ALT/SGPT		21	U/L	5-35
ALKALINE PHOSPHATASE		100	U/L	45-116
TOTAL PROTEINS		5.4 *	g/dL	6.02-8.0
ALBUMIN		3.1 *	g/dL	3.7-5.6
GLOBULINS		2.3 *	g/dL	2.5-3.0

End of the report


Lab Technician

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DR. JATESH KG

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CIN : U085100KA2012PTC065541

Verified By

Laboratory Test Result

Register No.: 24178964

Patient Name: KAMALAKAR GANAPATHI HEGDE

Dept/Unit: OTORHINOLARYNGOLOGY/Unit 2

Report No.: L3600940

Age: 80 Sex: MALE

Location: OPD Day



Bill/Advice No : 2450468833

Collection Date & Time : 25-02-2025 10:27 AM

Lab ID/Slide No : S/967/2025

Sample Received Date & Time: 25-02-2025 10:27 AM

Report Status: Final

HISTOPATHOLOGY

Histopathology*

CLINICAL DETAILS :

Proliferative growth over base of tongue.

? Malignancy base of tongue.

GROSS :Received multiple (6) pale white to dark brown soft tissue bits altogether measuring 0.7x0.4x.2cm.
All embedded in 967.MICROSCOPY: Report date 27/2/25

Section studied show tissue bits lined by dysplastic stratified squamous epithelium with few groups of atypical squamous cells invading the subepithelium. These cells have high N:C ratio, prominent nucleoli & eosinophilic cytoplasm. Increased mitosis with atypical ones are noted. Dense peritumoral inflammatory infiltrates noted.

IMPRESSION:

Features are suggestive of Poorly differentiated Squamous cell Carcinoma - Base of tongue.

End of Report

Dr. Kishan Prasad M.D. (Path)
Professor of PathologyDr. Shruthi, M.D(Path)
Asst Professor of Pathology

XXXXX

Printed On: 04-03-2025, 10:04
Report Generated On: 01-03-2025, 04:33

Note: Tests marked with * symbol are not covered under the scope of NABL.



HUBLI SCAN CENTRE

6-13, LGF, Eureka Junction, Travellers Bungalow Road, HUBLI - 580 029.

☎: 0836-2257828
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Patient Name:	KAMALAKAR HEGDE 83y/M	Patient ID:	118689
Age:	83 Years	Sex:	M
Study Date:	28-Jul-2025	Referred By:	Dr. VISHWAS D. PAI M.S., M.Ch., DNB.(Onco)

C.T. NECK AND CHEST

Plain and Contrast enhanced MDCT Scan of the Neck and chest was performed.

Clinical Information: A known case of CA base of tongue with right cervical lymph node metastasis

The Study Reveals -

A large ill-defined enhancing ulceroproliferative mass lesion is noted in right side of base of tongue. Lesion is extending into right vallecula and right tonsillar fossa. Lesion is involving left side of base of tongue, median and right glossoepiglottic folds, right side of epiglottis, preepiglottic space, right aryepiglottic fold and right lateral pharyngeal wall. Lesion is abutting right cornu of hyoid bone. No bony erosion is seen. Focal extension into left aryepiglottic fold also noted. Narrowing of airway is noted. No extension into pre-vertebral space noted. Lesion approximately measures 4.8 x 3.7 x 3.3 cm.

Multiple enlarged metastatic lymph nodes are noted in right level II, III and V B. Lymph nodes are discrete as well as confluent. Largest lymph node measuring 4.5 x 3.3 cm is noted in right level II and parapharyngeal space. It is encasing right internal carotid artery and external carotid artery and its branches causing luminal narrowing. Severe compression of right internal jugular vein is noted at the level of right level II and III lymph nodes.

Small lymph nodes are noted in left level II. They are most likely reactive in nature.

Oral tongue appears normal.

Oral cavity is normal.

Floor of mouth is normal.

Nasopharynx is normal.

Pyriform sinuses are normal.

Glottic and infra glottic larynx are normal.

Thyroid is normal.

Eccentric soft plaque is noted in right carotid bulb and at the origin of right internal carotid artery causing 70% stenosis. It measures 4 mm in length.

Parotid and submandibular salivary glands are normal.

Paranasal sinuses are clear.

Hypertrophy of bilateral inferior turbinates is seen.

Deviation of nasal septum towards right side is seen.

Both orbits are normal.

Visualised brain parenchyma is normal.

-cont'd.-

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Consultant Radiologist

Dr. Rajeev B. Dibbad
M.D., D.N.B.
Consultant Radiologist

Dr. Raghavendra
D.M.R.D., M.I.
Consultant Radiologist

Radiology Report

Register No.: 24178964

Patient Name: KAMALAKAR GANAPATHI HEGDE

Bill/Advice No.: 2450469343

Referred By: -

Report No.: R253843

Age: 80 Sex: MALE

Observed Date & Time: 25-02-2025 11:42 PM

Doctor: -



NECK

History: C/o dysphagia, odynophagia. VLS - Growth in base of tongue extending to lingual surface and involving the left glosso-epiglottic fold.

Protocol: Plain and Contrast CT scan of the neck was done by taking 5mm Sections from thoracic inlet to the base of skull. 3D reformations done and viewed in soft tissue and bone windows.

Findings:

Ill defined irregular heterogeneous lesion measuring 37x32x45mm noted epicentered in the right vallecula and epiglottis involving the medial and right glosso epiglottic fold.

The right vallecula appears obliterated and lingual surface of the epiglottis and extending to the base of the tongue. Right lingual tonsillar sulcus appears obliterated. Tonsillar fossa and pharyngo tonsillar pillars appears not involved by the lesion. The pre epiglottic fat appears involved by the lesion.

Posterior extension to the posterior pharyngeal wall at the epiglottis level. Posteriorly the fat planes with prevertebral muscles appears ill defined. Superiorly the lesion extends upto the superior cornua of the hyoid bone. Inferiorly reaching upto the superior part of the the right aryepiglottic fold.

Lesion mildly crosses the midline however left glosso epiglottic fold appears not involved. There is narrowing(>50%) the supraglottic lumen of air way.

Laterally lesion confined by the body of the hyoid bone. No hyoid bone erosions seen.

Multiple enlarged conglomerate lymph nodes seen in the right cervical levels II & III, largest in level II largest measuring 44x26x50mm with non enhancing necrotic areas within. It is seen abutting the right carotid space, compressing the right IJV.

The oral cavity appear normal.

The nasopharynx appear normal.

The subglottic region appears normal.

Post cricoid and cervical esophagus appears normal.

Imaged portions of the parotid and submandibular glands show no abnormalities.

The thyroid gland appears symmetrical, normal in size and internal structure.

Left cervical vessels have normal appearance.

Degenerative changes noted in the Visualised spine. Few lytic areas noted in the few vertebrae.

Visualized lung shows panacinar emphysematous changes noted. Dependent atelectatic changes noted.

Right inferior turbinate hypertrophy noted.

IMPRESSION:

- Ill defined irregular heterogeneous lesion in right vallecula and epiglottis extending to base of tongue and associated cervical lymphadenopathy as described. Radiological staging corresponds to T3N2Mx
- Degenerative changes in the Visualised spine with few lytic areas.
